



Laboratory Investigation Report

Patient Name	: Rinku Singh	Centre	: 1533 - Arctern Healthcare South Delhi
Age/Gender	: 40 Y 0 M 2 D /F	OP/IP No	: OP/110325
Max ID/Mobile	: 486648/8752005200	Collection Date/Time	: 03/Nov/2021 07:57AM
Lab ID	: 1265112100014	Receiving Date	: 03/Nov/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 03/Nov/2021
Passport No.	:		

Test Name	Immunoassay Result	Unit	Bio Ref Interval
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Anti Mullerian Hormone, Serum*

Anti Mullerian Hormone (AMH) CLIA	1.12	ng/mL	0.00 - 3.27
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Ref Range Interpretation :

Anti-Mullerian Hormone (AMH) is a hormone secreted by cells in developing egg sacs (follicles). The level of AMH in blood is generally a good indicator of ovarian reserve.
Low AMH levels are considered to be an indicator of a **low ovarian reserve**, i.e. few remaining follicles. AMH levels decline with age, and in younger women this may be a sign of premature loss of fertility
AMH does not change during menstrual cycle, so the blood sample can be taken at any time of the month - even while using oral contraception.
AMH level for a fertile woman is 1.0-4.0 ng/ml

In males AMH is secreted by immature Sertoli cells (SC) and is responsible for the regression of Müllerian ducts in the male fetus as part of the sexual differentiation process. AMH is also involved in testicular development and function.

AMH level ng/ml	Effects for fertility treatment
<0.4	Very low value. Very few eggs at stimulation. Pregnancy chances significantly low.
0.4 – 1.0	Low value. Treatment may be possible.
1.0 – 3.5	Normal value. Good possibility of treatment.
>3.5	Suggestive of ovarian hyperstimulation syndrome / PCOS

Note :- Optimal ovarian reserve values range between 2 - 6 ng/mL in reproductive age group

Kindly correlate with clinical findings

*** End Of Report ***



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SIN No: B2B1051632, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

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Max Super Speciality Hospital, Saket (West Block), 1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050
(CIN No.: U89100DL2021PLC381826)

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Laboratory Investigation Report

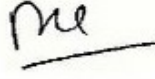
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Immunoassay

Test Name	Result	Unit	Bio Ref Interval
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Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



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Clinical Biochemistry

Blood Sugar Fasting, Fluoride Plasma

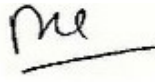
Date	03/Nov/2021 07:57AM	Unit	Bio Ref Interval
Glucose (Fasting) Hexokinase	92	mg/dL	74 - 99

Kindly correlate with clinical findings

*** End Of Report ***



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Immunoassay

Prolactin, Serum

Date	03/Nov/2021 07:59AM	Unit	Bio Ref Interval
Prolactin CLIA	29.18	ng/mL	

Ref Range

Males :	2.64 - 13.13
Females :	
Premenopausal (<50 years of age):	3.34 - 26.74
Postmenopausal (>50 years of age):	2.74 - 19.64

Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery
Decreased in pituitary apoplexy (Sheehan's Syndrome)



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Immunoassay

TSH,Serum

Date	03/Nov/2021 07:57AM	Unit	Bio Ref Interval
Thyroid Stimulating Hormone	5.50	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

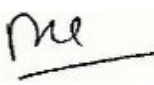
Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

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